

Goals of care discussions are conversations that happen between patients/caregivers of chronic illness with poor long term outcome and their treating physician. It is a clinical note documenting the values of the patient/caregivers including their wishes for further care - Including decisions on admission to the ICU, requirement of ventilator, or feeding tube, and so on.

Patients with advanced malignancy frequently present to the emergency department unable to communicate their treatment preferences. Although goals-of-care discussions may previously have been done, the documentation is often incomplete, unstructured, outdated or difficult for emergency clinicians to retrieve quickly. Thus, families may be asked to repeat distressing conversations, while clinicians make critical decisions without reliable knowledge of the patient's values, designated surrogate or previously discussed limits of treatment.

We propose a physician-led digital goals-of-care record that converts a clinical conversation into a concise, structured and patient-authorized summary. The record would be verified by the treating physician, remain updateable and version-controlled, and be easily accessible in times of emergency. The emergency physician would then be able to spend less time on these conversations and can resort to simply verifying the contents of this document. It would inform emergency decision-making while clearly distinguishing a goals-of-care summary from more ethical or legal heavy documents such as advance medical directives, do-not-resuscitate orders, or decisions to withdraw life-sustaining treatment.

The creator of the document would be the primary oncologist/palliative medicine physician, the user would be the emergency medicine physician, and any long term care provider would be able to update this document.

The primary outcomes will be the time taken to retrieve the record and document an appropriate treatment plan. Secondary outcomes will include caregiver anxiety, repetition of goals-of-care discussions, record utilisation and alignment of emergency care with the patient's documented preferences.